

KEY FIGURES OF JAN VISHWAS-I

42

**NUMBER
OF ACTS
AMENDED**

183

**TOTAL
PROVISIONS
DECRIMINALISED**

19

**MINISTRIES/
DEPARTMENTS
INVOLVED**

~110

**OFFENCES
CONVERTED TO
CIVIL PENALTIES**

MAIN OBJECTIVE

EASE OF DOING BUSINESS BY REPLACING JAIL TERMS FOR MINOR OFFENCES WITH MONETARY PENALTIES

NEXT IN LINE (JAN VISHWAS 2.0)

OVER 100 ADDITIONAL LAWS UNDER REVIEW BY DPIIT; PROPOSAL EXPECTED IN 2025 WINTER SESSION



using a Government Analyst's report in advertising drugs or cosmetics—which earlier attracted a ₹5,000 fine—may now invite a penalty of up to ₹1 lakh. Repeat offences will attract a minimum ₹5 lakh fine; there will be no jail time. They say the law expands the scope for compounding offences under Sections 27(d) and 27-A(ii), allowing monetary settlements instead of criminal trials. But imprisonment isn't removed entirely—it remains for serious violations like spurious drugs or unlicensed operations. They caution that the impact will depend on how state regulators implement these changes and whether companies treat penalties as deterrents and not operational costs.

Pharma companies, which face scrutiny from multiple regulators, have long objected to criminalising technical lapses. So, offences like late data submission or minor label errors will now attract fines, not jail.

“While we welcome decriminalisation, the bigger issue remains the multiplicity of regulators and outdated compliance rules,” says a



“This Act sends a strong message that compliance should be encouraged through collaboration, not coercion. Even minor technical lapses in economic offences attracted criminal penalties. That needed to change”

PINKY ANAND

Senior advocate and former additional solicitor general

senior pharma industry executive. “A trust-based regime is good, but clarity and predictability are equally important,” he adds.

Hospital chains and healthcare providers say implementation is patchy across states. “States are the implementing authority for most health regulations. Unless they align with the Centre's trust-based approach, the impact will remain limited,” says Girdhar J. Gyani, Director General, Association of Healthcare Providers (India).

This is especially true of mid-sized and regional diagnostic labs. For example, enforcement over missing consent forms or biomedical waste records is common. “India's diagnostic sector has grown rapidly, especially with mid-sized and regional labs stepping in to serve smaller towns and cities,” says Dr. Harsh Mahajan, Chair, FICCI Health Services Committee. “While this is a welcome shift, it also brings regulatory challenges.” He adds: “Enforcement actions in recent months show that even labs trying to follow the